



MaryCatherine McDonald Ph.D.
The Hope Circuit

ADVERSE CHILDHOOD EXPERIENCES

Why Your ACEs Score Doesn't Mean As Much As You Might Think

Defining trauma with ACEs risks overlooking other factors or misinterpreting their impact.

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✓ Reviewed by Devon Frye

KEY POINTS

- The ACEs framework's reductionist view on trauma risks pathologizing and stigmatizing individuals.
- We need to move beyond ACEs to a holistic, trauma-informed approach that incorporates positive experiences.
- An events-based approach to defining trauma overlooks other adversities and/or misinterprets their impact.

In the mid-1990s, the concept of Adverse Childhood Experiences (ACEs) was introduced through a study conducted by Kaiser Permanente and the Centers for Disease Control and Prevention. This study was designed to see if there were any long-term health impacts that could be traced back to traumatic childhood events like abuse, neglect, and various household challenges.

Since then, the ACEs framework has gained widespread recognition. It has been adopted by various organizations and professionals as a quasi-diagnostic tool that promises to understand the consequences of childhood trauma. It has also reached the public, with many readily available online quizzes that can tell you your “ACEs score” within minutes. When a tool becomes so widespread in this way, we often take its veracity for granted and stop thinking critically about whether it is effective.

But, *is* your ACEs score meaningful? I argue that the answer is no—at least, not in the way that it is usually presented.

While the initial study undoubtedly made valuable contributions to our understanding of the lasting effects of adverse experiences in early life, the over-emphasis on ACEs as a primary lens through which we view childhood trauma is not only misguided but also harmful. It is time to reevaluate our reliance on this framework and consider alternative approaches that better capture the complexity of human experiences.

One of the most significant limitations of the ACEs study is its narrow focus on a specific set of ten adverse experiences. This is problematic because while this framework could certainly present a list of *potential* traumatic events, we need to move away from the idea that it is possible to make a list of potentially traumatic events. The major thing that determines whether any given event creates lasting symptoms of trauma is the nervous system of the person living through that event.

Using an events-based definition of trauma risks misunderstanding clients in both directions. On the one hand, by narrowing our attention to these ten experiences, we risk overlooking other potentially damaging events or circumstances that can profoundly impact a child's development and well-being. While these experiences may indeed be traumatic, they do not encompass the full range of childhood adversities that individuals may face.

On the other hand, these experiences may be present in a child's life but *not* experienced as traumatic if they occur simultaneously with other protective factors. For example, parental separation or divorce *may* be traumatic but in and of itself is not *inherently* traumatic. If a child has a robust social support network and the separation is amicable and not contentious, there may be no lasting effects on mental health or otherwise.

Moreover, the ACEs framework wildly oversimplifies the relationship between adverse experiences and later outcomes. While the study established correlations between ACEs and negative health and social outcomes, it is crucial to recognize that *correlation does not imply causation*. The human experience is inherently complex, and a multitude of factors, including individual resilience, social support systems, environmental factors, and access to resources, *all* influence how individuals navigate and cope with adverse experiences over the course of their life.



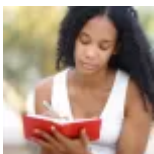
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Another significant concern with the ACEs framework is its potential to perpetuate a deficit-based perspective on childhood trauma. By primarily focusing on adverse experiences, and creating a score-based hierarchy of trauma, we risk pathologizing individuals who have endured such events and overlooking their inherent strengths and resilience. This deficit-based approach reinforces stigma and further marginalizes those who have experienced adversity.

There are also study design limitations that should not be ignored. For example, the ACEs study was conducted on a predominantly white (71 percent), middle-class population, which should raise questions about its applicability to diverse cultural and socioeconomic contexts. Trauma and adversity can manifest differently across various communities and backgrounds, and a one-size-fits-all approach fails to capture the nuances and complexities of these experiences.

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And it wasn't until 2019 that researchers acknowledged that it is crucial to recognize the role of *positive* experiences in mitigating the effects of trauma. The more recent study by researchers at Johns Hopkins University on Positive Childhood Experiences (PCEs) highlights the protective factors that can foster resilience and promote positive outcomes even in the presence of adversity. By incorporating a strengths-based perspective that acknowledges both adverse and positive experiences, we can develop more comprehensive and effective interventions that empower individuals and communities.

To truly address the lasting impacts of childhood trauma, we need a paradigm shift that moves well beyond the limitations of the ACEs framework. This shift should involve a

more holistic and culturally sensitive approach that recognizes the multidimensional nature of adversity and resilience.

One potential avenue is to adopt a trauma-informed approach that acknowledges the widespread impact of trauma and emphasizes the importance of creating safe, supportive, and inclusive environments throughout development. This approach recognizes that trauma can manifest in various forms, not just the specific adverse experiences outlined in the ACEs study, and that individuals may respond differently to traumatic events based on their unique circumstances and contexts.

Additionally, we should prioritize community-based interventions that empower individuals and families to identify and build upon their existing strengths and resources. By fostering supportive networks and promoting collective resilience, we can create environments that mitigate the impact of adverse experiences and nurture positive childhood experiences.

Ultimately, our understanding of childhood trauma and its impacts has evolved. It is time to move beyond the limitations of the ACEs framework and embrace a more comprehensive, strengths-based, and culturally responsive approach. By doing so, we can better support individuals and communities in overcoming adversity, promoting resilience, and fostering positive outcomes for all.

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About the Author



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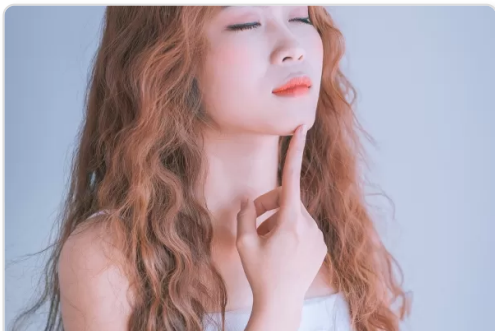
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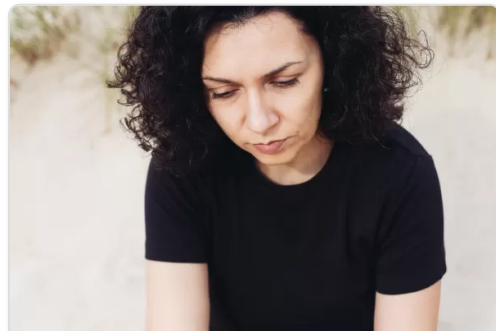


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